

ST. JUDE METROPOLITAN HOSPITAL

Department of Endocrinology & Internal Medicine • Tel: (555) 019-2834

CLINICAL CONSULTATION REPORT

Patient Name:	Eleanor Vance	Record Number:	MRN-92841-A
Date of Birth:	October 14, 1974	Age / Gender:	51 / Female
Date of Visit:	June 08, 2026	Physician:	Dr. Robert Chen, MD

CHIEF COMPLAINT

Patient presents for a follow-up evaluation regarding a 6-month history of progressive fatigue, polydipsia (excessive thirst), and polyuria (frequent urination). She reports an unintentional weight loss of approximately 8 lbs over the past two months despite an increased appetite.

HISTORY OF PRESENT ILLNESS (HPI)

Mrs. Vance is a 51-year-old female with a sedentary lifestyle and a family history of metabolic disorders. She reports that her symptoms began gradually but have intensified over the last several weeks, significantly impacting her sleep patterns due to nocturia (3-4 times per night). She denies any acute chest pain, shortness of breath, or localized neurological deficits, but notes occasional transient blurred vision and mild bilateral lower extremity paresthesia (tingling in toes).

PHYSICAL EXAMINATION

Vital Signs: BP: 138/88 mmHg | HR: 76 bpm | Temp: 98.4°F | Respiratory Rate: 16 bpm | BMI: 31.4 (Obese Class I).

General: Alert, oriented, in no acute distress. Appears mildly fatigued.

Cardiovascular: Regular rhythm, normal S1/S2, no murmurs or gallops.

Extremities: Trace bilateral pedal edema. Decreased sensation to light touch and monofilament testing over the plantar surfaces of both feet symmetrically. Arterial pulses (dorsalis pedis) are 2+ bilaterally.

LABORATORY FINDINGS

Test Panel / Parameter	Result	Reference Range	Status
Fasting Plasma Glucose	168 mg/dL	70 – 99 mg/dL	High
Hemoglobin A1c (HbA1c)	8.2 %	4.0 – 5.6 %	High
Serum Creatinine	0.92 mg/dL	0.60 – 1.10 mg/dL	Normal
Total Cholesterol	224 mg/dL	< 200 mg/dL	High
Triglycerides	195 mg/dL	< 150 mg/dL	High

ASSESSMENT & DIAGNOSIS

Primary Diagnosis: Type 2 Diabetes Mellitus (ICD-10: E11.9) - newly diagnosed, currently uncontrolled.

Secondary Diagnoses: Hyperlipidemia (ICD-10: E78.5), Essential Hypertension (ICD-10: I10), and Early Peripheral Diabetic Neuropathy.

MANAGEMENT & TREATMENT PLAN

1. **Pharmacotherapy:** Initiate Metformin HCl 500 mg orally twice daily with meals. Titrate to 1000 mg twice daily in two weeks if GI tolerance allows. Prescribe Atorvastatin 20 mg orally once daily at bedtime for lipid optimization.
2. **Lifestyle Modification:** Referred to a Certified Diabetes Educator (CDE) for medical nutrition therapy. Restrict simple carbohydrates, increase dietary fiber, and initiate a low-impact cardiovascular exercise routine (e.g., walking 30 minutes, 5 days per week).
3. **Monitoring:** Patient provided with a blood glucose meter; instructed to check fasting and 2-hour postprandial glucose levels twice daily.
4. **Referrals & Follow-up:** Scheduled a formal comprehensive ophthalmology exam for diabetic retinopathy screening. Return to clinic in 3 months for repeat HbA1c and basic metabolic panel.

Dr. Robert Chen, MD

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Department of Endocrinology